PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Evaluating communication with parents in paediatric patient-encounters – A Systematic review protocol
AUTHORS	Cusson, Olivier; Mercier, Justine; Catelin, Céline; Zomahoun, Hervé Tchala Vignon

VERSION 1 – REVIEW

REVIEWER	Zanello, Marc Saint Anne Hospital Centre
REVIEW RETURNED	26-Feb-2021

GENERAL COMMENTS	The authors should be thanked for their protocol entitled "Evaluating communication with parents in pediatric difficult patient-encounters – A Systematic review protocol". This topic is really crucial for medicine improvement. They tried to provide a well-detailed and well-designed protocol, following the COSMIN and PRISMA guidelines. However, some limitations restrict the study' strengths. The authors should provide a revised version of the manuscript. Major comments: Page 8, Lines 9-11. "We chose 16 years old because we believe the older adolescent's parent's role in consultations is different than in younger patients". I disagree with this single threshold. The child development could be divided in many stages of growth but the main theories distinguish before and after 10 years-old. I would consider apart the study with children from birth to 10 years-old and children from 10 to 16 years-old. Page 8, Lines 33-34. "We will exclude interactions taking place in a telehealth context (encounters over the phone, over web platform, over videoconference, etc..)" and 48-55 "This protocol was first conceptualized before the COVID-19 pandemic and at that time, telemedicine saw very little use in practice, although it is now expected to gain significant importance. We believe we would find very little literature regarding the rating of communication in telemedicine in a pediatric setting, and that many criteria for measuring communication quality in face-to-face interviews will be applicable for telemedicine". Page 12, Lines 40-46. "This review was constructed during the COVID-19 pandemic and will not be addressing encounters over a telemedicine setting. We believe this literature is prone to expand greatly in the next couple of years as an increase in usage of

	telemedicine technologies is currently seen all around the world. It would be interesting as a subsequent study to review the literature existing the evaluation of communication quality over a telemedicine setting. This review's methodological construct and content will regardless be of use to whoever may take interest in this topic." It is now impossible to ignore the pandemic situation and its impact on telemedicine. Moreover, as stated by the authors, the literature will probably be scarce. This limitation should be overcome by including the encounters taking place in a telemedicine context. In fact, this review can help to identify the tools able to appreciate quality of telehealth interviews. Minor comments: Page 4, line 21. "and or validated": and/or validated? Page 4 line 22. "residents, physicians": residents and/or physicians? Page 5 line 15 "guidDPelines": guidelines? Page 7, Line 40 to Page 8, Line 34. Consider to rephrase with complete numbering "1) the construct; 2) the population(s); 3) the type of instrument(s); 4) the measurement properties of interest; and two other elements 5) study designs; 6) the settings in which the studies have to be conducted" for instance and use numbered list rather than bulleted list. Page 10, Line 28. "a third person will be consulted for a final decision". Please detailed this person (CC?). Page 12, Line 18. "be disseminated the team's" : be disseminated to the team's?

REVIEWER	Kasat, Kavita Northwell Health, Pediatrics
REVIEW RETURNED	03-May-2021

GENERAL COMMENTS	Overall, the protocol is well designed and follows COSMIN and PRISMA guidelines. The following concerns should be addressed: 1. The objective is to "identify validated tools for measuring communication skills or communication effectiveness with parents in a pediatric setting in general, including for difficult encounters." However, based on the "Eligibility Criteria" (specifically page 8, lines 37-47) it seems that the plan is to just look at difficult encounters. The primary objective should not include communication in general if that is not the focus. 2. As such, the outcomes need to be more clearly defined. Are you just looking at difficult encounters? Or tools for all encounters? 3. There are several grammatical errors: a. Abstract- line 21 - "residents and physicians" b. Throughout the paper, replace "and or" with "and/or" c. Page 5, line 14 - "guidDPelines" should be corrected

	d. Page 6, line 10 - replace "encounters is" with "encounters are" e. Page 6, line 21 - I am unclear as to what "medical formation" means f. Page 6, line 52 - "Commmunication's quality perception" is confusing. g. Page 10, lines 43 and 44 - replace "number of men" with "male/female ratio" h. Page 10, line 56 - remove "etc" or expand on what you mean.

VERSION 1 – AUTHOR RESPONSE

Response to reviewer 1 (Dr Marc Zanello):
Thank you for taking the time to review our protocol. Your suggestions will help strengthen our work and we appreciate it.

Regarding the cutoff of 16 years old of 80% of the study population in the eligibility criteria: We further specified our reasoning for that cutoff. It should be noted that in the analysis, we will be very interested to consider the different age groups the tools were used in because, as you mentioned, different approaches should be considered depending on the age of the child. We chose to not analyze samples mostly of patients beyond 16 years old because these patients will be interviewed without their parents often and our study's aim is towards communication with the patient's parents.

Regarding the analysis of consultations taking place over a telehealth setting, we recognize that it is a field that has taken a lot of expansion in the last year. Unfortunately, the selection process is already underway our team's financial and time resources would not allow us to go over the screening of 5000 articles again. I am fairly certain that no major literature exists on the subject after having gone through the articles myself. We also believe that many of the aspects of good doctor-parent communication will be the same whether the context be in telehealth or in face to face interviews – the telehealth setting would be adding on some criteria. Finally, our review aims at furthering knowledge of communication in difficult patient encounters. We believe breaking bad news and difficult encounters should take place in face to face interviews most of the time.

For the minor comments, all comments were addressed. Thank you

Response to reviewer 2 (Dr. Kavita Kasat)
Thank you for your comments and suggestions, they will surely help us strengthen our protocol's quality.

Regarding the clarification of the study's objectives, we made appropriate changes to clarify. We initially wanted to focus only on difficult patient encounters, but quickly realized that defining a difficult patient encounter was very subjective and doing such would risk ignoring assessment tools that could potentially be relevent. We chose not to exclude any encounters based on the setting of the interview / the difficulty of the encounter, but to rather comment on that aspect during our analysis. Our main outcome of interest is still to identify tools appropriate for difficult patient encounters, but those modifications have made it so we will also identify tools used in a general setting.

Regarding the grammatical mistakes, they have all been addressed. Thank you.

VERSION 2 – REVIEW

REVIEWER	Zanello, Marc Saint Anne Hospital Centre
REVIEW RETURNED	07-Jun-2021

GENERAL COMMENTS	The authors adequately answered the suggestions. In particular, the explanation of the 16 years-old threshold is now clearer. The study focusing on difficult encounters, it is understandable to not include telehealth setting.

REVIEWER	Kasat, Kavita Northwell Health, Pediatrics
REVIEW RETURNED	15-Jun-2021

GENERAL COMMENTS	Thank you for addressing prior concerns and revising the manuscript. The clarification in the text that you will be including all patient encounters is helpful. However, I would recommend changing the title of your manuscript. You have stated that your objective will be to look at all encounters, not just difficult ones. Consider changing title to "Evaluating communication with parents in pediatric patient encounters: A systematic review protocol".

VERSION 2 – AUTHOR RESPONSE

Response to Dr Kavita Kasat:
- We have changed the title as suggested to better reflect the precisions we brought to the manuscript.